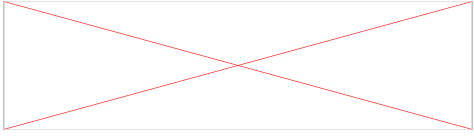




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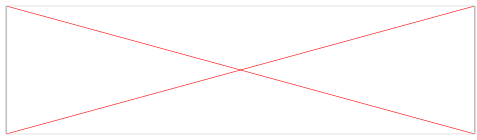
Genital Prolapse

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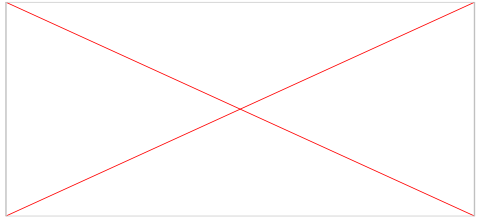
objectives

- Understand the anatomy of supporting ligaments and fascia of the female pelvic organs.
- Appreciate the relationships between anatomical prolapse and functional symptoms, including urinary, bowel and sexual dysfunction.
- Learn how to assess the patient with prolapse by means of history, examination and relevant investigations.
- Understand the principles of treatment of prolapse and be able to describe the effectiveness of each treatment.



Definition and impact:

- pelvic organ prolapse (POP) define as "...falling, slipping or downward displacement of the uterus and/or the different vaginal compartments and their neighboring organs such as bladder, rectum or bowel.
- Although not life-threatening, it has a significant impact on the quality of life, can affect perception of body image and may cause depressive symptoms..



Symptoms from pelvic organ prolapse

Pelvic organ prolapse can cause symptoms directly due to the prolapsed organ or indirectly due to organ dysfunction secondary to displacement from the anatomical position.

Prolapse symptoms include a sensation of vaginal bulge, heaviness or a visible protrusion at or beyond the introitus. Patients may also describe lower abdominal or back pain, or a dragging discomfort relieved by lying or sitting.

- Indirect symptoms will depend on which other organs are involved in the prolapse but may include difficulty in voiding urine or emptying the bowel (termed obstructive defaecation), and sensations of incomplete emptying of bladder or rectum.
- Patients may have to support or reduce the prolapse with their fingers to be able to void or evacuate stool completely (termed digitation). Urinary or faecal incontinence may also be present

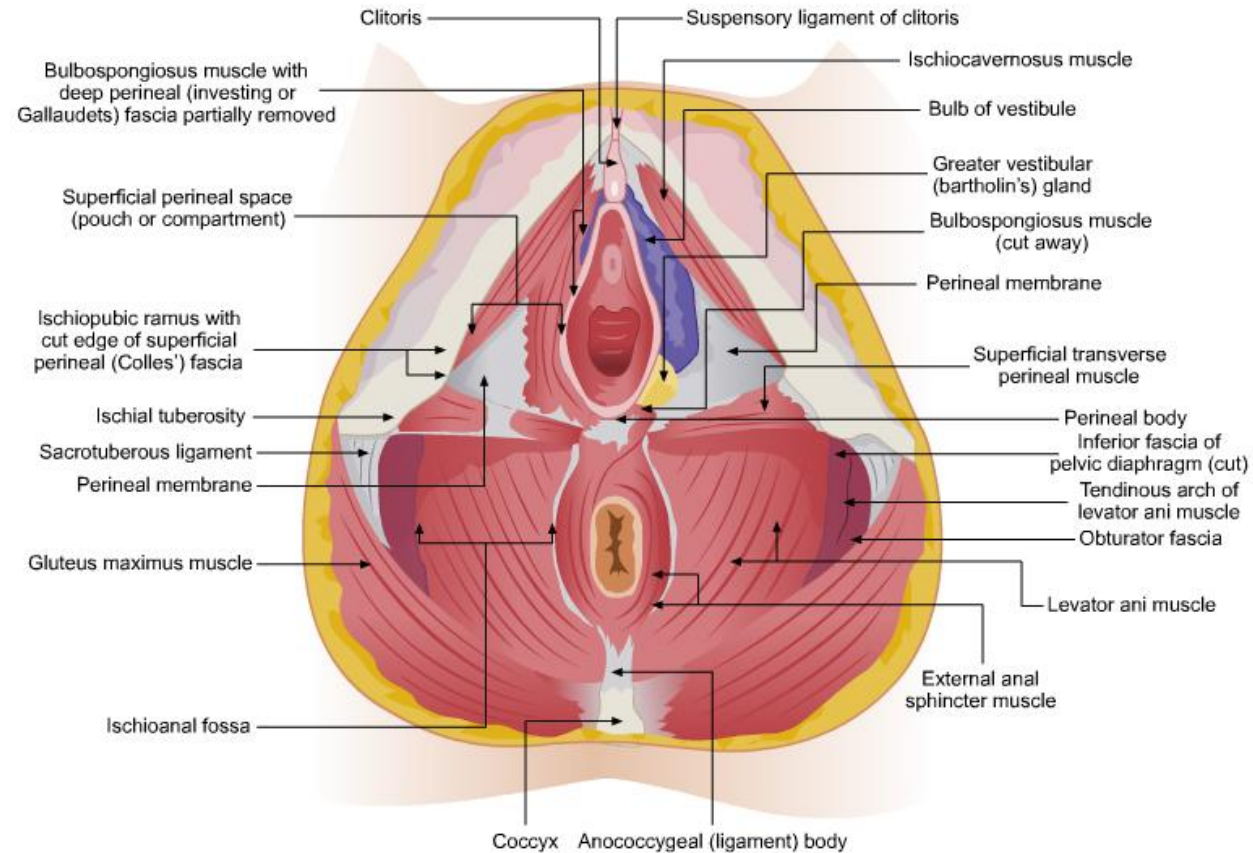
- It is important to ask about sexual activity, even in the older patient, and to enquire about difficulty achieving penetration, pain or discomfort during intercourse and loss of sensation and difficulty achieving orgasm due to vaginal or introital laxity.
- Patients may experience vaginal bleeding from a prolapse that is external and becomes ulcerated or abraded, but in women with a uterus, one must remember to exclude endometrial carcinoma by biopsy and ultrasound

Risk factors predisposing to prolapse are very much similar to those predisposing to stress incontinence.

Research has shown the pudendal nerves to be damaged after childbirth, with increased nerve conduction times, and ultrasound studies of the pelvic anatomy of women with prolapse have demonstrated thinning or avulsion of the puborectalis muscle from its insertion on the pubic ramus, on either one or both sides in a high proportion of cases

Relevant anatomy

- The levator ani muscles (puborectalis, pubococcygeus and iliococcygeus) support the pelvic organs and relieve excessive pressure from the ligaments and fascia.
- The uterosacral ligaments provide essential apical support (level 1 support).
- Vaginal fascia supports the vagina (level 2 support).
- The perineal body is very important in supporting the lower vagina (level 3 support).
- All the structures provide a dynamic and integrated support to the pelvic organs.



Clinical assessment of prolapse

The history should elicit the presenting symptom(s) and severity, and include questions to ascertain if the patient has any coexisting urinary, faecal or sexual symptoms .

One should be sensitive to the emotional aspect of the problem, but specific questions should be asked about sexual discomfort and difficulty achieving orgasm.

Clinical examination should ideally be done in the lithotomy position with a Sims speculum .

This allows retraction of the anterior and posterior vaginal wall in turn, to allow full assessment of the degree of prolapse and to assess how much descent of the cervix and uterus is present.

Prolapse is described in **four stages** of descent, and note should be made of whether it occurs at patient straining or at rest and whether traction has been applied:

- Stage 0 –no prolapse is determined
- Stage I the most distal portion of the prolapse is more than 1 cm above the level of the hymen.
- Stage II -the most distal portion of the prolapse is 1cm or less proximal to or distal to the plane of the hymen.
- Stage III-the most distal portion of the prolapse is more than 1cm below the plane of the hymen.
- stageIV-complete eversion of the total length of the lower genital tract is demonstrated.

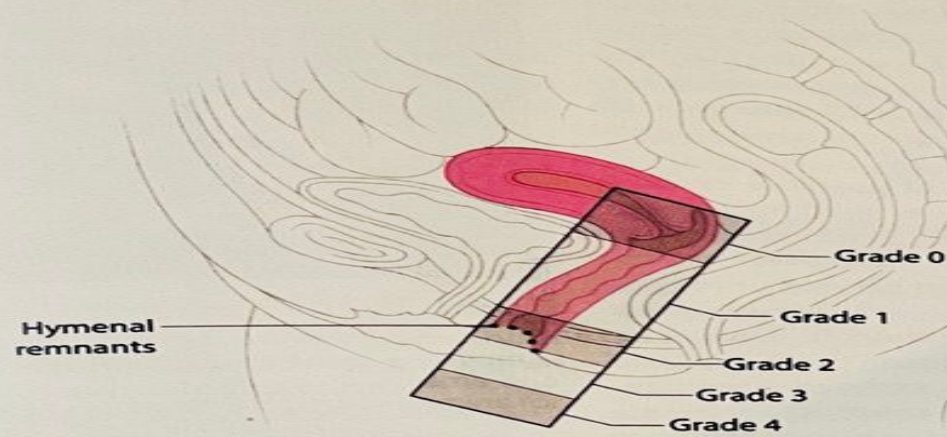


Figure 10.10 Pelvic organ prolapse staging system.

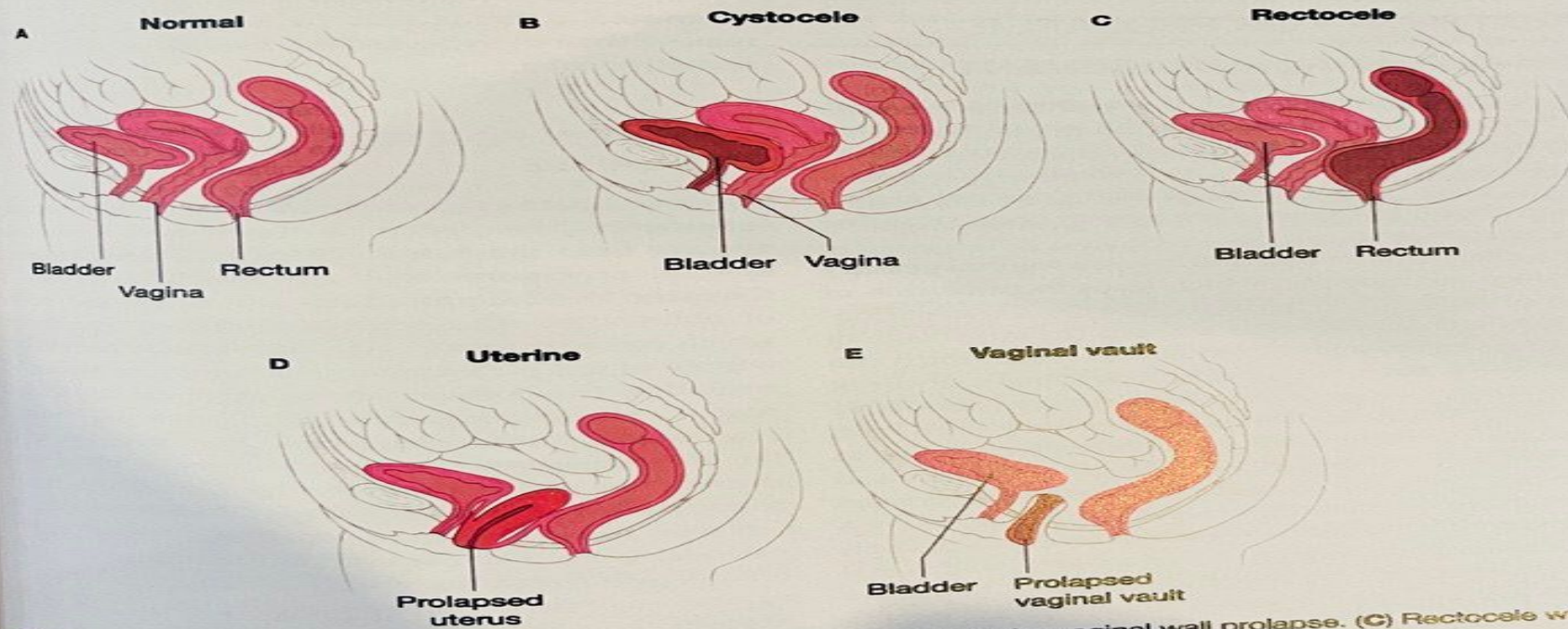


Figure 10.11 (A) Anterior vaginal wall prolapse. (B) Cystocele with Anterior vaginal wall prolapse. (C) Rectocele with Posterior vaginal wall prolapse. (D) Uterine prolapse. (E) Vaginal vault prolapse.

In women who have undergone hysterectomy, the vaginal vault can prolapse .

Vaginal prolapse of the anterior vagina (the anterior compartment) is also known as **cystocele** in the upper half or **urethrocele** in the lower half.

Posterior vaginal prolapse (the posterior compartment) is also known as **enterocele** in the upper third, or **rectocele** below this

Treatment for prolapse

•Non surgical management

Conservative treatment for prolapse includes pelvic floor muscle exercises and the use of supportive vaginal pessaries.

A programme of supervised PFMT for at least 4 months for individuals with symptomatic pelvic organ prolapse that does not extend greater than 1 cm beyond the hymen upon straining is recommended.

- An alternative to this is to insert a vaginal support pessary to reduce the prolapse, which leads to resolution of many of the symptoms. Pessary use can be very effective at relieving symptoms and has the advantage of avoiding surgery and the associated risks, which can be extremely useful in the medically unfit and elderly.
- A range of shapes of pessary is available . Ring pessaries are usually tried first, but an intact perineal body is necessary for these to be retained. Shelf pessaries, Gelhorn pessaries and others are useful for women with deficient perineal bodies. It is usual practice to replace a pessary every 6 months and to examine the patient for signs of vaginal ulceration.



Surgery for pelvic organ prolapse

Surgical treatment for prolapse is common, and can be offered if conservative treatments have failed or if the patient chooses surgery from the outset.

The procedure chosen depends on which compartment is affected, whether the woman wishes to retain her uterus and whether the vaginal or abdominal route of surgery is chosen.

The essential principles of prolapse surgery apply for all procedures. Prolapse surgery is performed through the vagina to restore the ligamentous tissue supports to the apex, anterior and posterior vagina (anterior repair, posterior repair) and repair of the perineal body.

The vaginal route can also be used for post hysterectomy vault prolapse, attaching the vaginal vault to the right sacrospinous ligament with non-absorbable or slowly absorbable sutures, but here an abdominal approach to perform a sacrocolpopexy is an option that will provide excellent, durable long-term cure.

In the last 3–5 years, there has been an increasing number of women wishing to avoid hysterectomy during prolapse surgery, so both sacrospinous fixation and sacrocolpopexy can be performed by attaching a mesh or sutures to the cervix rather than the vaginal vault.

Principles of prolapse surgery

- Remove/reduce the vaginal bulge.
- Restore the ligament/tissue supports to the apex, anterior and posterior vagina.
- Replace associated organs in their correct positions.

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- Retain sufficient vaginal length and width to allow intercourse.
- Restore the perineal body.
- Correct or prevent urinary incontinence.
- Correct or prevent faecal incontinence.
- Correct obstructed defaecation.

- Anterior vaginal repair (colporrhaphy):for symptomatic anterior vaginal prolapse not a procedure for stress incontinence
- Sutures to reinforce fascia between vagina and bladder.
- Risk of bladder injury and urinary retention ,relatively high recurrent rate.

- Posterior vaginal repair (colporrhaphy):for posterior vaginal prolapse can improve obstructed defaecation
- Sutures to reinforce fascia between vaginal and rectum
- Risk of rectal injury ,with post operative dyspareunia.

conclusion

- Although prolaps is not life-threatening, it has a significant impact on the quality of life.
- Clinical assessment of prolapse is by history and examination.
- Prolapse is described in **four stages** of descent.
- Conservative treatment for prolapse includes pelvic floor muscle exercises and the use of supportive vaginal pessaries.
- Surgical treatment for prolapse is common, The procedure chosen depends on which compartment is affected.